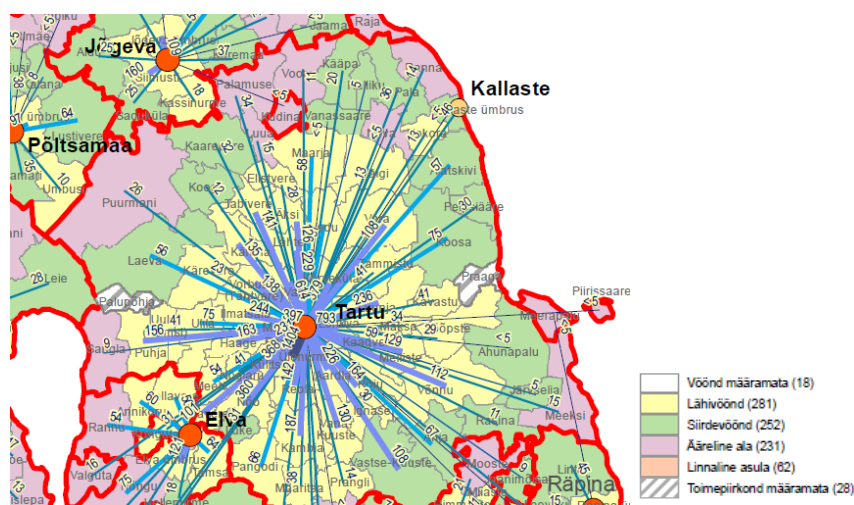


Ekspertarvamus Luunja valla haldusterritoriaalse korralduse võimaliku muutmise kohta

Luunja vald ei täida haldusreformi seadusega pandud miinimumsuurusekriteeriumi 5000 elanikku, seisuga 01.01.2017 oli Luunja vallas registreeritud 4251 elanikku.

Luunja vald kuulub Statistikaameti analüüsi¹ põhjal Tartu linna lähivööndisse (joonis 1).



Joonis 1. Tartu toimepiirkond. Allikas: Statistikaamet (2014) Toimepiirkondade määramine

Luunja vald kuulub Tartu linnapiirkonna koosseisu ja on paljudes eluvaldkondade toimimises seotud Tartu linnaga. Vallas on 20 küla ja üks alevik. Rahvaarvult suurim on Lohkva küla. Kohalikeks tömbekeskusteks küladele on Lohkva, Luunja ja Kavastu, mis on endisaegsete ühismajandite keskused. Intensiivne ehitustegevus on valda toonud hulgaliselt uusi elanikke, rahvastikuregistri andmetel on elanike arv aastatel 2007-2015 kasvanud enam kui 1000 inimese võrra. Luunja valla arengut mõjutab oluliselt Tartu valglinnastumise protsess. Märkimisväärne osa rahvastikust elab Lohkva (1181 elanikku) ja Veibri (577) külates, mis asuvad Tartu linna administratiivpiiril. Linnast kaugemale jäävates külates asustustihedus oluliselt langeb ja tegu on pigem maalise elulaadi viljelemisega.

Luunja valla ühendamisel Vabariigi Valitsuse algatusel tuleb kaaluda ettepaneku esitamisel kahe võimaluse vahel – ühendamine Tartu vallaga või Tartu linnaga. Asjakohane ei ole Luunja valla ühendamine Haaslava, Mäksa ja Võnnu valdade baasil moodustuva uue omavalitsusüksusega, mis on märksa maalisema iseloomuga kui Luunja vald ning puudub kogemus ja ressursid Tartu valglinnastumise käigus Tartu linnapiiril kujunenud tiheasustuse piirkondade arendamiseks.

Tartu linnaga ühendamise poolt on järgmised asjaolud:

- Tartu linnaga ühendatakse linnaga kokku kasvanud Lohkva ja Veibri külade tiheasustusalad Luunja vallast viies need ühtse juhtimise alla ja moodustub loogiline

¹ Statistikaamet (2014) Toimepiirkondade määramine.

planeerimispiirkond. Samas maaliste Luunja valla alade arendamine eeldab keskuslinna käsitlest erineva linnastu kontseptsiooni rakendamist.

- Luunja valla Lohkva ja Veibri külates elab ligi pool Luunja valla elanikkonnast, kes saavad ühinemisel Tartu linnaga võimaluse rääkida kaasa arengutes, mis mõjutavad nende tegelikku igapäevast teenuste tarbimist. Olukord, kus elanikud ei saa oma elukeskkonda maksumaksjana mõjutada ja selle arengus kaasa rääkida, ei ole demokraatlik.
- Ühise omavalitsusena on liikumiskeskond paremini korraldatav, et tagada ennekõike keskuslinnas Tartus töökohtade, teenuste ja vaba aja võimaluste kättesaadavus praeguse Luunja valla elanikele.
- Tekivad paremad võimalused läbi planeerimise arendada Tartu linna. Lohkva ja Veibri küla ühinemisel moodustuks loogiline Tartu linna tervikala.

Tartu vallaga ühendamise poolt on järgmised asjaolud:

- Ühinemisel Tartu vallaga saab Vabariigi Valitsuse algatatud ühendamise käigus pakkuda terviklahendused Tartu valla kujundamiseks (arvestades ka teisi ühinevad omavalitsusi) luues parima elukeskkonna ja lahendused elanikele ühises toimealas.
- Tartu vald ja Luunja vald on arengus sama sotsiaal-majandusliku iseloomuga omavalitsused, neil on analoogsed arenguprobleemid ja väljakutsete lahendamine. Neil on vahetud Tartu valglinnastumise mõjuga piirkonnad, sh sisuliselt Tartu linnaga kokku kasvanud alad ning samas on valdadel ka linnalähedase maapiirkonnana Tartu linna vahetust mõjust väljapoole jäävad lähivööndi alad (kuigi ka nendes piirkondades on Tartu linnal suur mõju võimekusega pakkuda töökohti, avalikke teenuseid ja võimalusi vaba aja veetmiseks).
- Luunja ja Tartu vald on teinud väga olulisi pingutusi, et tugevdada omavalitsuste terviklikkust ja sisemist sidusust. Seega on olulisel kohal olnud mõlemas omavalitsuses nii linnalise kui maalise elulaadi toetamine. Sarnasuse tõttu on juhtimiskultuuride ühendamine lihtsam.
- Luunja vallavolikogu on teinud vabatahtliku ühinemise etapis ettepaneku ühinemiseks Tartu vallaga (kes küll keeldus esitatud ettepanekust).

Role of Ayurveda in Dry Eczema

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INTRODUCTION:

~~———— In Ayurveda, various skin disorders are detailed under the heading Kushtha. It is one of the Mahagada defined by Ayurveda which not only affects the skin but also other dhatus for example Rakta, Mansa etc. Vicharchika is specially mentioned under the heading of Kshudra Kushtha (minor skin ailments). Even though, in terms of severity, incidence and prognosis it is not minor. Vicharchika, a type of Kshudra Kushtha (chronic skin disease) though involves only epidermis, its tendency of recurrence continues to pose problems to the~~

physician.Characteristics of Vicharchika as mentioned in the Brihat trayi are Atikandu, Shyavata, Pidika, Bahustrava or Rukshata and Raji. The consideration of characteristics of Vicharchika as Eczema may also reveal the same fact.

1. ~~Atikandu (excessive itching) In eezema, itching varies from mild to severe paroxysm which even may interfere with work and sleep.~~
 2. ~~Bahu strava (lasikadhya) Eezema is characterized by superficial inflammatory oedema of epidermis associated with vesicle formation. In acute condition wet eezema is common~~
 3. ~~Rukshata (dryness) Chronic illness of Eezema also persists or it may be found in the same patients at different stages.~~
 4. ~~Shyaava Pidika In chronic cases of eezema, integument appears thickened and is hyperpigmented.~~
 5. ~~Raji (Lichenification) Due to scratching in chronic cases of eezema, the skin becomes thick, hyperpigmented with visible criss cross ridges known as lichenification.~~
- ~~Considering the fact that, the combination of external as well internal therapy will play its best role in cure of Eezema. With this view, for present research I have proposed to focus the upashayatmaka effect of Aragwadhadi Kashaya (kalpit compound) as internal medicine and Yashadamrut malahar as external application in patients of Vicharchika.~~

~~NEED OF STUDY:~~

- ~~❑ Incidence Great prevalence of the disease i.e. 3% in Indian Population~~
- ~~❑ Burning problem Severe itching disturbing routine and its nature susceptible to be chronic.~~
- ~~❑ Socio Psychological impacts Affects psychological status and disturb the social life due to its appearance,~~
- ~~❑ WHO celebrates 29th October as World Psoriasis day~~

~~AIMS:~~

- ~~❑ To study Nidanpanchaka (aetiopathogenesis) of Vicharchika and its Upashaya according to Ayurveda.~~
- ~~❑ To study the Upashaytmaka effect of Aragwadhadi Kashaya and Yashadamrut malahara lepa on Vicharchika.~~

~~OBJECTIVES:~~

- ~~❑ To assess the efficacy of Aragwadhadi kashaya and Yashadamruta Malahara in the management of Vicharchika w.r.to Eezema.~~
- ~~❑ To draw a hypothesis regarding the mode of action of drugs used in the present research on the scientific basis.~~

MATERIALS AND METHODS:

1. Conceptual Study

Disease Review: It comprises of

- 1) Ayurvedic review
- 2) Modern review

1) Ayurvedic review: In this, Vicharchika will be defined according to Ayurvedic literature

2) Modern review: In this, Eczema will be defined according to Modern literature

Following materials are used for this study.

- ☐ **Selection of Patient** Minimum 60 patients having signs and symptoms of “Vicharchika” were randomly selected from OPD and IPD wings of Government Ayurved College, Nanded
- ☐ The patients will diagnosed on the basis of classical signs and symptoms of Vicharchika described in Ayurvedic classics like Kandu, Daha, Strava, Rukshata, Pidika, Vaivarnya, Raji and Vedna etc.
- ☐ **Informed written consent** was taken
- ☐ **Type of study:** Clinical trial
- ☐ **Study design:** Parallel group
- ☐ **Patients selection:** Randomized
- ☐ **Examination of Patient** Examination of Patient was done according to special case proforma which was included
 - **Etiological factors of Vicharchika** with
 - **Dushti lakshan of Dosha,**
 - **Dushya,**
 - **Strotas etc.**
- ☐ **Resulting Pathogenesis or Samprapti after administration of hetu** was studied in detailed.

INCLUSION CRITERIA-

1. Patients showing Pratyatma Lakshana of Vicharchika (eczema) described in classical Ayurvedic texts as well in modern medical literature.
2. Age group of 16 to 60 years of either sex

~~3. Chronicity of disease not more than 3 years.~~

~~—EXCLUSION CRITERIA—~~

~~1. Patients suffering from disease like Anemia, Hemorrhagic disorders, Hemiplegia, Convulsion, Tuberculosis, Cancer; severe systemic diseases like Diabetes, Renal disease, H/O liver diseases etc.~~

~~2. Pregnancy, lactation~~

~~Registered 60 patients were divided in two groups as-~~

~~1. Group A: Control group~~

~~———30 registered patients of Vicharchika were administered Tablet Defcort in the dose of 6 mg twice a day with a glass of water for 30 days.~~

~~2. Group B: Aragvadhadi Kashaya~~

~~———30 registered patients of Vicharchika were administered with Aragvadhadi Kashaya (Kalpita compound) in the dose of 40ml twice a day for 30 days along with Yashadamruta Malahara lepa at night as external application on affected parts of body.~~

~~Tablet Defcort :~~

- ~~☐ Dose- 6 mg twice a day with a glass of water after food,~~
- ~~☐ Duration- at same time every day for 30 days.~~

~~Aragvadhadi Kashaya-~~

- ~~☐ Period: 4 weeks~~
- ~~☐ Dosage: 40 ml twice a day with parts of Ghrita~~
- ~~☐ Duration: Morning 40ml and Evening 40ml (daily)~~
- ~~☐ Bhaishajya Kal: Before meal~~
- ~~☐ Mode of Administration: Oral~~

~~Yashadamruta Malahara-~~

- ~~☐ Period: 4 weeks~~
- ~~☐ Dosage: Quantity sufficient as per affected area of body~~
- ~~☐ Period of application: In day time~~
- ~~☐ Mode of Administration: External application~~

~~LABORATORY INVESTIGATIONS—~~

~~☐ Routine investigations wherever needed~~

~~Actual Plan of Work-~~

~~☐ The patients were diagnosed on the basis of classical signs and symptoms of Vicharchhika with the help of special standard case paper incorporating Nidanpanchaka~~

~~☐ More concentration was given on Evaluation of Hetus and resulting Samprapti or Nidanpanchaka.~~

~~☐ After diagnosis of the patients, study was intervened by Upashayatmaka therapy(exploration) in the form of Aragvadhadi kashaya and Yashadamruta Malahara~~

~~☐ Group A: Control Tab.Defcort 6mg twice a day~~

~~☐ Group B: Aragvadhadi Kwatha: 40ml twice~~

~~☐ Patients were studied for 6 weeks with the follow up after every 1 week interval.~~

~~☐ The Upashayatmaka effect of Aragvadhadi kashaya and Yashadamruta Malahara was assessed in regards to the clinical signs and symptoms on the basis of grading and scoring systems and overall upashaya.~~

~~☐ **Place of work:** OPD and IPD Wings of Government Ayurved College, Nanded~~

~~☐ **Formulation of drug :** Was carried out in Rasashala, Government Ayurved College, Nanded~~

~~☐ **Duration of work:** Was completed in scheduled course of 4 week~~

~~Criteria of assessment:~~

~~☐ Criteria of assessment were kept on the basis of relief in the signs and symptoms of the disease Vicharchhika.~~

~~☐ For this, the cardinal signs and symptoms were given scores according to their severity, before and after treatment.~~

~~1. Kandu(Itching)~~

~~2. Daha (Burning sensation)~~

~~3. Vaivarnya (Discoloration)~~

~~4. Strava (discharge)~~

~~5. Raji (Lichenification, thickening and ridges on skin)~~

~~6. Rukshata (Dryness)~~

7. Pidika (Eruption)

8. Vedana (Pain)

Contents of Aragvadhadi Kashaya (Kalpita compound)

S. N.	Contents	Botanical name	Part used	Quantity
1	Aragwadha	Cassia fistula	Root	1 Part
2	Nimba	Azadiracta indica	Root	1 Part
3	Guduchi	Tinospora cordifolia	Kanda	1 Part
4	Khadir	Acacia catechu	Root	1 Part
5	Kutaja	Hollarhena antidysentrica	Root	1 Part
6	Atrushaka	Adhatoda vasica	Root	1 Part

Contents of Yashadamruta Malahara (Rasa tarangini)

S. N.	Contents	Botanical name	Quantity
1.	Siktha	Wax	12 gm
2.	Tilataila	Sesamum indicum	q.s.
3.	Yashad	Zincum	1 tula

Praman of lepa— $\frac{1}{4}$ angul $\approx$ 5mm

GRADING AND SCORING OF SIGNS AND SYMPTOMS—

Grading and Scoring—

For symptoms———subjective parameters were considered

For signs———appropriate clinical parameters were considered

Scoring—

Grade III———+++———severe

Grade II———++———moderate

Grade I———+———mild

Grade 0———0———nil

1) Kandu (itching)–

+++——itching continuously and cannot concentrate on
———routine work and in Night

++——intermittent itching for more than 5 times per hour

+——itching only once or twice a day

0——no itching

2) ~~Daha (burning sensation)~~

- ~~+++ continuously burning cannot concentrate on work and even during sleep~~
- ~~++ burning sensation but can concentrate on work~~
- ~~+ burning only 1 or 2 times a day~~
- ~~0 no burning~~

3) ~~Vaivarnya (discolouration)~~

- ~~+++ easily looking discolouration~~
- ~~++ mild discolouration only visible in light~~
- ~~+ very mild discolouration only visible in light~~
- ~~0 no discolouration~~

4) ~~Srava (discharge)~~

- ~~+++ continuous discharge patient requires dressing to lesion to soak up the discharge and disturb his routine work~~
- ~~++ need to put dressing but not disturbing~~
- ~~+ need not to put dressing~~
- ~~0 scanty discharge need not to soak or dressing and dries their own~~

5) ~~Raji~~

- ~~+++ multiple lines with discomfort~~
- ~~++ lines without discomfort~~
- ~~+ single line~~
- ~~0 absent~~

6) ~~Rookshata (dryness)~~

- ~~+++ with discomfort on skin~~
- ~~++ without discomfort on skin~~
- ~~+ negligible~~
- ~~0 absent~~

7) ~~Pidika (eruption)~~

~~Number of Pitika per square cm on the effected skin will be measured.~~

~~8) Ruja (pain)-~~

- ~~+++ continuously paining or aching cannot concentrate on~~
~~work and even during sleep~~
- ~~++ pain is there but can concentrate on work~~
- ~~+ pain only 1 or 2 times a day~~
- ~~0 no pain~~

~~OVERALL EFFECT OF THERAPY:~~

~~1. Cured:- 100% relief in the sign and symptoms with plain skin surface and significant changes in the color of the affected skin lesion towards normal was considered as cured.~~

~~2. Complete Remission:- more than 76 to 99% relief in the sign and symptoms was recorded as complete remission with marked improvement in the pigmentation and~~

Observation	Result	Total % n=20
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~~thickening of the skin.~~

~~3. Marked improvement:- 51-75% relief in sign and symptoms was considered as marked improve with in pigmentation and thickening of the skin.~~

~~4. Improvement:- patients showing improvement between 25-50% in sign and symptoms with slight improvement of pigmentation and thickening of the skin was taken as improvement.~~

~~5. No relief:- below 25% relief sign~~

~~OBSERVATIONS:~~

~~On the basis of results found during study upashayatmaka effect of Aragvadhadi kashaya and Yashadamruta Malahara was quoted as follows-~~

	31-40yrs	56
	Female	31
	Hindu	88
	Urban	95
Socio-economic	Middle	80
Education	Educated	86
Marital status	Married	76
Occupation	Housewife	26
Diet	Mix	86
Agni	Manda	41
Prakruti	Vata-pitta	40



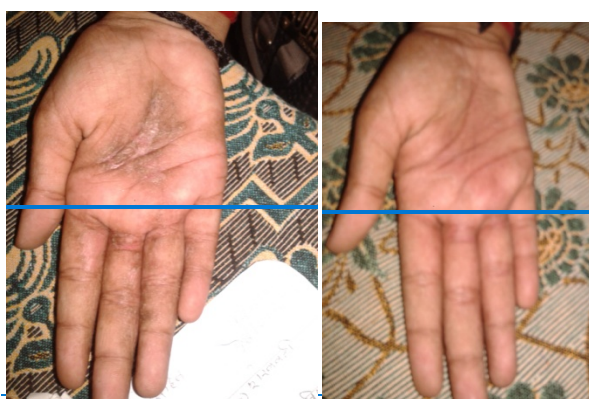
Formed Malahara

Contents of Yasahdamruta Malhara and Aragadhadi kashaya

RESULT

Symptoms	Group A % (n=10)	Group B % (n=10)
Kandu	53	86
Ruja	50	75
Vaivarnya	50	77
Daha	50	88
Strava	20	83

Rukshata	58	81
Raji	77	80
Pidika	0	0
Average	<u>53</u>	<u>81</u>



Before Treatment — **After Treatment**



~~Before Treatment ————— After Treatment~~

~~CONCLUSION:~~

- ~~■ Combined effect of Aragwadhadi Kashaya with Yashadamruta malahara was found more effective in patients of Group B(81%) than that of Group A patients treated with Tab. Defeort (53%)~~
- ~~■ Aragwadhadi Kashaya, a concentrated Decoction of herbal drugs showed mild laxative and detoxifier action.~~
- ~~■ Yashadamruta malahara showed Shodhan property; reduction and thickness of skin by exfoliation and scrapping, ropan and rejuvenation~~
- ~~■ More rate of recurrence observed with control group than of study group was nil, observed in follow up study~~
- ~~■ Economical; Cost effective~~
- ~~■ Yashadamruta Malahara provided better relief in thick dispigmented lesions of dry Eczema~~
- ~~■ Drug used in present research, proved their best effect against Vicharchika, in their combination form.~~

~~REFERENCES:~~

- ~~□ sa kNDU ipiDka Syaavaa bahUs~avaa ivacaica ka È ca.ica.7À26~~
- ~~□ rajyaa AitkNDU Ait\$jaa sa \$xaa Bavaint gaa~oYau ivacaica kayaama\ È sau.ina.5À137~~
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- ~~• Prepared trial product for present study is not a coomercial or Industry product.~~
- ~~• There are no any conflicts of interests.~~